

POST-PANDEMIC MENTAL HEALTH RECOVERY IN ENGLAND

NHS Talking Therapies: Executive Summary

Analysis Period: February 2019 to April 2026

EXECUTIVE SUMMARY

England's NHS Talking Therapies programme has not fully recovered from the impact of COVID-19. Five years after the pandemic began, clinical recovery rates remain below pre-pandemic levels despite a significant rebound in referral volumes and improved waiting time performance. This analysis examines post-pandemic recovery trajectories across 7 regions and 79 CCGs, stratified by deprivation, using NHS England open data covering February 2019 to April 2026.

KEY FINDINGS

1. Recovery rates have declined and not recovered

The national recovery rate fell from 51.79% pre-pandemic to 50.14% post-pandemic, a decline of 1.65 percentage points that has persisted throughout the five-year recovery period. The reliable recovery rate, a stricter clinical measure, fell further from 49.18% to 47.20%, a decline of 1.98 percentage points. Approximately 1 in 2 patients completing a course of treatment does not achieve clinical recovery.

2. Demand has rebounded strongly but outcomes have not kept pace

Monthly referrals averaged 148.13K in the post-pandemic period, exceeding pre-pandemic levels of 141,694. Access has improved, with 90.28% of patients now treated within 6 weeks compared to 87.68% pre-pandemic. The service is reaching more people more quickly, but clinical outcomes have not recovered proportionally.

3. Significant regional variation persists

North East and Yorkshire leads post-pandemic recovery at 51.20%, while the North West lags at 48.85%, a gap of 2.35 percentage points. Both London (49.93%) and the North West fall below the national average of 50.14%, suggesting structural challenges that predate the pandemic.

4. Deprivation is associated with lower recovery rates

During the pandemic period, the least deprived areas (Decile 1) achieved a recovery rate of 54.07% compared to 49.88% in the most deprived areas (Decile 10), a gap of 4.19 percentage points. This inequality pattern predates the pandemic and represents a persistent structural challenge for commissioners.

5. Predictive modelling identifies 37 high-risk CCGs

A Gradient Boosting classifier (ROC-AUC: 0.69) identified 37 of 79 assessed CCGs as high risk for below-average post-pandemic recovery. Pre-pandemic baseline performance (34% feature importance) and waiting time performance (32%) are the strongest predictors of post-pandemic outcomes, suggesting that areas that struggled before COVID-19 are most at risk of continued underperformance.

RECOMMENDATIONS

- **Targeted intervention in high-risk CCGs:** The 37 CCGs classified as high risk, particularly those in the most deprived deciles, require commissioner-level review and targeted support.
- **Investigate the North West:** The region's persistent underperformance warrants a root cause analysis examining workforce capacity, case-mix complexity, and access barriers.
- **Deprivation-adjusted benchmarking:** National recovery rate targets should be adjusted for population deprivation to ensure equitable performance assessment across commissioners.
- **Monitor the volume-outcomes tension:** Rising referral volumes alongside declining recovery rates suggests the service may be treating higher volumes at the cost of outcome quality. This requires ongoing monitoring.

LIMITATIONS

- Deprivation linkage is restricted to pre-pandemic and pandemic periods due to the 2022 CCG to ICB restructure, which created a geographic discontinuity in the data.
- Predictive model was trained on 79 CCGs only, representing those with complete matched data across pre-pandemic and post-pandemic periods, limiting generalisability across all English CCGs.
- IMD 2019 deprivation data does not reflect deprivation changes post-pandemic.

- Recovery rate data reflects patients who completed treatment, not all referrals, introducing potential selection bias.
- The wide format data structure of the pre-pandemic file (February 2019 to February 2020) differs from subsequent publications, which may introduce minor inconsistencies in CCG-level comparisons.

DATA AND METHODOLOGY

Data sourced from NHS England Talking Therapies monthly publications (February 2019 to April 2026), linked to the English Indices of Multiple Deprivation 2019 via LSOA-to-CCG geography. Analysis conducted in Python using Pandas and Scikit-learn. Findings visualised in Power BI and an interactive AI agent built using the Claude API and Streamlit.

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Data sources: NHS England, DLUHC, ONS Geography Portal

GitHub: github.com/aridunnu/nhs-mental-health-recovery

Live Agent: nhs-mental-health-recovery-2026.streamlit.app